

INVESTOR CONCEPT NOTE

MUMNEST

Pregnancy to baby's first year, connected to your doctor. An affordable digital care companion for India's middle-income families.

THE CORE BET

India already has pregnancy content and tracking apps. The gap is trusted daily support linked to the woman's own doctor - without turning the doctor into a 24x7 WhatsApp helpline.

~26.5M

Annual births, directional

Derived from 2023 SRS birth rate x population [1][2]

88.6%

Institutional births

NFHS-5 India [3]

₹150-200

Platform fee / patient / month

Founder pricing hypothesis

1. The problem we are solving

For many middle-income families, pregnancy care happens in short doctor visits every few weeks, followed by a long gap at home. Women - especially first-time mothers in nuclear families - face daily questions on food, supplements, movement, fetal development, reports, warning signs, labour preparation, breastfeeding and newborn care. Premium maternity hospitals and expert-led programs can address this, but are often outside the intended customer budget.

At the same time, independent gynecologists and maternity homes face the opposite problem: patients want continuous access, but routine questions arrive through calls and WhatsApp, creating workload without a structured workflow, audit trail, triage layer or clear monetisation.

2. Who is the initial user - and who pays?

END USER	BUYER / DISTRIBUTOR
Middle-income pregnant woman First-time or low-support mother; working or homemaker; digitally reachable; values trusted guidance but cannot justify premium wellness spend.	Independent gynecologist / maternity home Typical consultation fee hypothesis: ₹500-700. Wants patient retention, better service differentiation, less repetitive messaging and an optional new revenue line.

Founder hypotheses to validate: consultation band, patient delivery budget and doctor willingness-to-pay. External sizing sources: [1]-[3].

MARKET + COMPETITION

A big market - but we are not building another tracker

The market already proves that women want digital pregnancy and parenting support. MumNest wins only if it is easier to trust, easier to adopt and more useful to the doctor than a normal consumer app.

\$40M India category revenue, 2023 Pregnancy tracking + postpartum apps [4]	\$142.3M Projected India revenue, 2030 19.9% CAGR, 2024-30 [4]	Postpartum Fastest-growing subsegment Grand View Research [4]
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3. What already exists

Player	What they do well	Primary model / strength	Gap vs. MumNest thesis
iMumz	Live yoga, nutrition, care manager, pregnancy program	Expert-led D2C pregnancy wellness [5]	Not built around the patient's own independent doctor
Mamify	Pregnancy, postpartum, newborn and expert programs	Expert-led programs; New Born Care+ listed at ₹1,799/month [6]	Consumer program; doctor-distribution is not the core wedge
Healofy	Pregnancy/baby tracking, 450+ topics, community and expert Q&A	Mass-market content + community + commerce [7]	Broad consumer experience; clinic ownership is not the core model
Mylo	Pregnancy + baby tools, growth, vaccines, community	10M+ users; broad free tools + commerce [8]	Strong reach; not a white-labelled doctor continuity system
Kilkari	Stage-based maternal/child preventive guidance	IVR + WhatsApp; 11 languages; public-health scale [9]	Excellent education reach, but not private-doctor SaaS
Premium hospitals	Integrated appointments, records, care pathways	Strong trust + clinical relationship	High-cost ecosystem; not available to every local clinic

4. The white space

THE GAP WE WANT TO OWN

Mass-market apps give information. Premium hospitals give connected care. MumNest aims to bring the best of both to neighbourhood gynecologists: affordable daily guidance for the mother, with her own doctor staying at the centre of the relationship.

5. Why this exact model can work

TRUST IS BUILT-IN The recommendation comes from the woman's own gynecologist, not an ad or influencer.	DOCTOR HAS A REASON TO SELL Better patient experience, stronger retention and an optional recurring margin.	ONE DOCTOR BRINGS MANY USERS A single doctor can add new pregnancy cohorts every month, reducing consumer acquisition cost.
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Sources: Grand View Research [4]; iMumz [5]; Mamify [6]; Healofy App Store [7]; Mylo App Store [8]; ARMMAN Kilkari [9].

PRODUCT

One app for the mother. One simple dashboard for the doctor.

The mother gets clear daily guidance. The doctor gets useful context and only the questions that actually need attention.

6. The core operating loop

KNOW	DO	TRACK	ASK	ESCALATE
What is happening?	What should I do today?	What is changing?	Is this normal?	When should my doctor step in?

7. Mother experience - MVP

Today Personalised week/day plan: nutrition, safe movement, hydration, supplement/medicine reminders, sleep and appointment nudges.	My Pregnancy / My Baby Fetal development, expected changes, movement education, scan/test timeline, postpartum recovery and baby milestones to 1 year.
Ask + Red Flags Medically reviewed education for routine doubts, structured symptom capture and clear routing: self-care information / contact clinic / urgent in-person care.	Track + Records Weight, BP/glucose where relevant, symptoms, kick logging, tests, prescriptions, reports, appointments, later feeding/growth/vaccinations.
Classes + Partner Mode Short vernacular video/audio routines, group sessions, labour preparation, breastfeeding basics and partner-specific tasks.	Tradition vs Science Respectful evidence checks for common nani/dadi advice: safe, avoid, or discuss with doctor.
Postpartum Mother recovery, lactation, emotional wellbeing, C-section/vaginal recovery education, nutrition and return-to-work routines.	Newborn to 1 year Holding, burping, colic/gas education, safe sleep, fever navigation, feeding, solids, teething, development and vaccination reminders.

8. Doctor dashboard - the real B2B product

PATIENT LIST Gestation, next visit, reported symptoms, adherence and recent uploads. Sort by review priority.	SMART INBOX Routine education stays in-product. Structured questions reach staff/doctor only when review is needed.	PRE-VISIT SUMMARY A concise timeline of vitals, symptoms, adherence, reports and questions so the consultation starts with context.
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9. Product roadmap

PHASE 1 - PREGNANCY Today plan, week-by-week journey, reports, reminders, Ask + red flags, doctor dashboard, white label.	PHASE 2 - POSTPARTUM + NEWBORN Mother recovery, lactation, newborn basics, feeding, sleep, colic education and pediatric handoff.	PHASE 3 - BABY TO 1 YEAR Growth, vaccines, milestones, solids, teething, development and pediatrician-led continuity.
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CLINICAL SAFETY

MumNest educates, tracks and escalates; it does not diagnose. Clinical decisions stay with registered medical practitioners, with privacy, consent and telemedicine requirements built into the workflow. [10][11]

BUSINESS MODEL

Doctor-led distribution makes the economics work

The clinic can absorb the fee, pass it on, or mark it up. In every case, the patient is acquired through a trusted doctor rather than expensive consumer advertising.

10. Base pricing hypothesis

₹150-200 MumNest fee Per active patient / month	₹299-399 Illustrative clinic price If doctor chooses markup	6-8 mo Typical paid pregnancy window Assumption for initial economics
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11. Three ways a doctor can commercialise it

Model	MumNest fee	Patient price	Doctor economics	Why doctor chooses it
ABSORB	₹150-200/mo	₹0 incremental	Clinic treats it as service cost	Differentiation, retention, premium patient experience
PASS THROUGH	₹150-200/mo	₹150-200/mo	No/low direct margin	Simple, transparent continuity-of-care add-on
MARK UP	₹150-200/mo	₹299-399/mo	~₹99-249 gross contribution / patient-month	New recurring revenue + stronger engagement

12. Example: one doctor, 100 active patients

₹17,500/mo MumNest bill 100 x ₹175	₹34,900/mo Clinic collects if priced at ₹349	₹17,400/mo Gross contribution before clinic service cost	₹2.10L Annual MumNest revenue from one 100-patient practice
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13. White labelling is part of the distribution strategy

The patient can experience “Dr. Mehta Pregnancy Care - powered by MumNest” or the maternity home’s own program. The clinic remains the visible relationship owner while MumNest supplies the digital operating layer.

- Doctor chooses free, bundled, pass-through or marked-up pricing.
- Clinic branding, welcome flow, appointment links and escalation rules stay configurable.
- No marketplace switching: the originating clinic remains the relationship owner.

WHY DOCTOR-LED ACQUISITION MATTERS

Win the doctor once -> new pregnancy cohorts every month -> 6-8 months of pregnancy revenue -> postpartum and pediatric continuation. Trust is reused instead of repurchased for every mother.

MARKET SIZE + GTM

How big can MumNest become?

A bottom-up view using published birth data and clearly stated assumptions on price, duration and doctor adoption.

14. TAM / SAM / SOM

Layer	Population basis	Annual software value	Definition / assumption
TAM	~23.4M institutional births	~₹2,872 Cr	~26.5M births inferred from SRS 2023 + population; 88.6% institutional; ₹175 x 7 paid months. [1][2][3]
SAM	~7.1M private-facility births	~₹866 Cr	Private institutional share approximated as 88.6% institutional minus 61.9% public = 26.7% of births. [3]
SOM	1,000 doctors x 75 active patients	~₹15.8 Cr ARR	Operational 3-year target example: 75,000 active patient-months billed monthly at ₹175. Not a forecast; a capture scenario.

15. Distribution and acquisition strategy

01 PROVE 20 doctors; 30-50 patients each. Two-city pilot. Validate paid continuation + activation.	02 PRODUCTISE White-label templates, staff onboarding, QR/WhatsApp invites, review queue and clinic analytics.	03 DISTRIBUTE Gyne associations, maternity networks, labs/diagnostics and city-level referral sales.	04 EXPAND Pediatric handoff, maternity-home chains, then employer/insurer bundles.
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16. What must be true before this is investable

DOCTOR PULL 10+ of first 20 doctors continue paid near ₹150-200 per active patient-month.	PATIENT HABIT >60% remain weekly active after week 4; Today / Ask / Track drive repeat use.	WORKLOAD BENEFIT Routine questions reduce or become faster to handle; escalation does not create 24x7 burden.
TRUST High activation when the experience carries the patient's own doctor/clinic brand.	UNIT ECONOMICS Provider CAC is recovered through recurring cohorts from the same doctor.	SAFETY Medical governance, escalation, consent, data controls and auditability work before scale.

WHY THIS CAN BECOME A LARGE BUSINESS

Doctor-led distribution can turn MumNest into the digital care layer for thousands of independent women's-health practices - then extend into postpartum, newborn and pediatric care.

SOURCES

[1] ORGI SRS Bulletin 2023 | [2] World Bank WDI India population 2023 | [3] MoHFW NFHS-5 | [4] Grand View Research - India pregnancy tracking & postpartum care apps | [5] iMumz | [6] Mamily | [7] Healofy App Store | [8] Mylo App Store | [9] ARMMAN Kilkari | [10] NMC Telemedicine Practice Guidelines | [11] MeitY DPDP Rules 2025